

In healthcare, we continued with multiple investors. In 2014, TPG came on board after three rounds of fund-raising. They encouraged us to think bigger. Temasek coming on board was another milestone. They brought long-term capital. TPG has now been with us for 11 years and Temasek for almost 10 years. Those partnerships helped us grow.

We never thought we were going to be the largest healthcare chain. We took one step at a time, with a lot of failures. We tried to acquire Fortis. Somebody outbid us. But that made us realise it was possible. We could attract capital, people were willing to back us, and we could think bigger.

We had another failed attempt. But finally, we acquired Columbia Asia. After that, we have been able not only to acquire hospitals but also to integrate them well. In many cases, acquisition is the easy part. What makes the difference is how well you execute. Very few hospital networks in the country have executed acquisitions and integration so well.

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Q: You made some acquisitions and failed to buy others. What did those experiences teach you about capital allocation and deal-making?

A: Our failures were more about capital. We just did not have the conviction to bid higher. Sometimes it is not a question of whether we were right or wrong. In hindsight, you might say we should have gone for it, but it is tough to say. At that time, everybody collectively felt it was the right price to pay. We must have the discipline to say “this is what we are going to pay” and it is fine if we lose it. As long as we are disciplined about an asset, we will be fine. We do not have to win at all costs, especially now that we are a public company.

Q: Has private equity strengthened healthcare or made it more expensive?

A: It is unfair to blame private equity for the rising cost of healthcare. That narrative may have emerged in the West where there is no growth in healthcare. So, the

only way to create value is reducing costs. In India, the focus has never been on cutting costs or increasing prices. It is about improving efficiency, growing the business and building scale through acquisitions. The narrative that private equity increases costs and reduces quality is wrong. Private equity investors want the best outcomes. They focus on governance and processes. Companies with private equity alongside promoters are often better run than companies run only by promoters because the limited capital available to a promoter sometimes leads to shortcuts.

Q: Hospital chains are expanding rapidly and competition for both assets and doctors has intensified. How does Manipal plan to stay ahead?

A: Competition has always been there. It was there five years ago. It was there ten years ago. There is more private equity and bigger chains now. The biggest differentiator is continuing to do what we have done in the past: attract and retain good-quality doctors. As your network grows, attracting doctors becomes easier. The brand also becomes an important factor.

Location is another important factor. Doctors look at whether the hospital is close to their residence. Our strategy is to go closer to where people, and hopefully doctors, are.

Q: What makes a hospital an attractive acquisition for Manipal?

A: We prefer a chain because you get a larger footprint. But it depends on where the network is located. Is it in an area where we do not have a presence? Does it add to our network?

Some buildings are old. We look at the cost of fixing them. If it is a running hospital, we look at whether we can attract good doctors and strengthen the areas where it is lacking.

Q: What will drive the group's next phase of growth over the next five to ten years?

A: Healthcare will continue to be our flagship business. Our education vertical is also very strong. We have a tech company that is doing well. The health insurance industry is another area I am excited about.

There is a huge amount of secular growth in that sector. As India becomes more prosperous, insurance becomes a must-have. We believe there are strong tailwinds in that sector. These are the three areas we are focused on. We have not thought about anything new.

We are looking at Tier-II and Tier-III cities. We have some hospitals coming up, probably more in Tier-II